ASSIGNMENT 5

INDIVIDUAL CASE SAFETY REPORT (ICSR)

1. Introduction

An Individual Case Safety Report (ICSR) documents a suspected adverse drug reaction (ADR) in a structured format. This report demonstrates the pharmacovigilance assessment of a published case of abacavir-associated hypersensitivity.

2. Case Identification

Case title: Hypersensitivity Reaction Associated with Abacavir Therapy in an Indian HIV Patient – A Case Report.

Source: Published case report, Journal of Clinical and Diagnostic Research, 2014.

3. Case Validity Assessment

The case fulfils the four minimum criteria required for a valid ICSR:

- Identifiable patient: 44-year-old male.
- Identifiable reporter: Published case report authors.
- Suspected drug: Abacavir.
- Adverse event: Hypersensitivity reaction with skin rash and systemic symptoms.

4. Patient Information

Parameter	Details
Age	44 years
Sex	Male
Medical condition	HIV-1 infection
CD4 count	3 cells/μL

Relevant history	Chronic alcohol use for 15 years

5. **Suspected Drug Information**

Parameter	Details
Drug name	Abacavir
Dose	600 mg
Route	Oral
Frequency	Not separately reported
Date started	05/02/2014
Date stopped	26/02/2014

6. **Concomitant Medication**

- Atazanavir – 300 mg
- Ritonavir – 100 mg
- Co-trimoxazole – 160 mg/400 mg

These medicines were reported in the patient's treatment regimen. Their exact administration frequencies were not separately specified.

7. **Adverse Reaction Details**

Reported reaction: Hypersensitivity reaction with maculopapular exfoliative skin rash.

Associated symptoms:

- Itching
- Headache
- Vomiting

- Abdominal pain
- Fatigue
- Elevated liver enzymes

Onset: Developed after initiation of the new antiretroviral regimen; the exact onset date was not clearly specified.

Management: Suspected medicines were withdrawn and symptomatic treatment with levocetirizine was given.

Outcome: Skin symptoms resolved within 2–3 days, and liver function tests improved within approximately 5 days.

8. **Clinical Narrative**

A 44-year-old male diagnosed with HIV-1 infection was started on an antiretroviral regimen containing abacavir, atazanavir and ritonavir. Following treatment initiation, he developed a hypersensitivity reaction characterised by skin rash, itching, gastrointestinal symptoms and elevated liver enzymes.

The suspected medicines were withdrawn, and symptomatic treatment was provided. The patient's skin symptoms resolved within a few days, with improvement in liver function tests. Atazanavir was later reintroduced without recurrence of the same skin reaction. Abacavir rechallenge was not performed.

9. **Causality Assessment – WHO-UMC**

WHO-UMC classification categories:

- Certain
- Probable/Likely
- Possible
- Unlikely
- Conditional/Unclassified
- Unassessable/Unclassifiable

Selected category: ***Probable/Likely***

Justification: The reaction occurred after initiation of the suspected treatment and is a recognised adverse effect of abacavir. Clinical improvement followed withdrawal of the suspected medicines. Atazanavir was later reintroduced without recurrence, while abacavir rechallenge was not performed. Therefore, the association with abacavir is assessed as

Probable/Likely.

10. **Severity and Seriousness Classification**

- **Severity**: Severe clinical reaction, considering the systemic symptoms and liver enzyme abnormalities.
- **Seriousness**: Medically important suspected reaction requiring clinical intervention. The report does not establish that hospitalisation was caused by the ADR.
- **Expectedness**: Expected, as abacavir-associated hypersensitivity is a recognised adverse reaction.

11. **Reporter Information**

Reporter names:

- Manju Janardhanan
- Mohan Babu Amberkar V
- Sudha Vidyasagar
- Meena Kumari K
- Sadhana N Holla

Institution: Kasturba Medical College, Manipal University, Manipal, Karnataka, India.

Note: This is a literature-based educational case simulation, not an actual regulatory submission.

12. **ICSR Processing Workflow**

- Receive and record the suspected ADR report (Day 0).
- Validate the four minimum ICSR criteria.
- Check for duplicate reports and identify missing information.
- Enter case details and code the reaction and medicines using MedDRA and WHO Drug Dictionary, respectively.
- Prepare the clinical narrative and perform medical and quality review.

- Submit the report within the applicable regulatory reporting timeline.

13. **Conclusion**

This case demonstrates the identification, documentation and assessment of a suspected abacavir-associated hypersensitivity reaction. The case was assessed as Probable/Likely using WHO-UMC criteria, highlighting the importance of accurate ADR reporting and pharmacovigilance monitoring.

14. **Reference**

Hypersensitivity Reaction Associated with Abacavir Therapy in an Indian HIV Patient – A Case Report. Journal of Clinical and Diagnostic Research. 2014

Link : https://pmc.ncbi.nlm.nih.gov/articles/PMC4225912/